



Adverse Drug Reaction and Clinical Alert Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All services	All disciplines

2. Purpose

This procedure supports a standardised process for communicating adverse drug reactions and clinical alerts across the Armada Kalamunda Group using the Patient Administration System (webPAS).

The following alert types are not covered by this document:

- Emergency Department Information System External Register alerts – please see the [EDIS External Register Alert Management Procedure \(EMD-PRO-0143-18\)](#)
- Mental health alerts – please see the [Psychiatric Services Online Information System Alerts Process \(v0.5\)](#)
- Micro alerts – please see the [Methicillin Resistant Staphylococcus Aureus \(MRSA\): Infection Prevention and Management Procedure \(AKG-PRO-0270-17\)](#)
- Other risk alerts including child at risk alert, possession of weapons risk alert and home visit security risks.

This procedure details the minimum, mandatory requirements to comply with the principles of the [WA Clinical Alert \(MedAlert\) Policy MP 0053/17](#).

3. Adverse drug reactions

Before medications/drugs are prescribed, dispensed or administered for a patient, it is important that the patient's adverse drug reaction (ADR) history and documentation is reviewed to prevent patient re-exposure to a drug which may lead to an adverse event.

At every encounter with the hospital (i.e. attendance to the Emergency Department, inpatient ward, hospital in the home type service or outpatient clinic) the patient should be interviewed to determine whether the patient has experienced any previous adverse drug reaction or allergic responses when taking medications /drugs.

3.1 Previous adverse drug reaction

If previous ADR are identified the following actions are required to be undertaken:

- Supply the patient with a red patient identification band (for inpatients only) – no other coloured patient identification band is to be used. Provide an explanation to the patient and/or carer regarding the rationale for applying the red arm band and address any questions they may have.

- Ensure that the ADR details are appropriately recorded in the patient's healthcare record, including on the inside front cover of each volume of the hardcopy healthcare record and on each Hospital Medication Chart (HMC).
- Assess if life threatening – complete Clinical Alert Notification form (MR Alert 2) for adding to webPAS.

See [Appendix 2](#) for flowchart.

3.2 New adverse drug reaction

Any ADR that occurs during a patient's admission, and has not been previously identified, should be reported to the patient's admitting medical officer with the following actions required to be undertaken:

- Supply the patient with a red patient identification band – no other coloured patient identification band is to be used. Provide an explanation to the patient and/or carer regarding the rationale for applying the red arm band.
- Provide the patient and/or carer with the [Adverse Drug Reaction Information Brochure](#) for consumers, or equivalent, that has been completed by the treating medical team or pharmacist; discuss any questions and/or concerns that the patient and/or carer may have.
- Record details of the ADR on an Allergy/Adverse Drug Reaction and/or Drug Sensitivity sticker inside the front cover of the patient's healthcare record (see [Appendix 5](#)). This should at a minimum include:
 - The generic name of the medication/drug implicated
 - The reaction which occurred
 - The date of the reaction (if known)
 - The person documenting the ADR must sign and date the record.
- Add a generic alert sticker on the outside front cover of the patient's healthcare record.
- Document the ADR details on every current HMC in the red Adverse Drug Reaction box. Attach an 'Adverse Drug Reaction' sticker on the back page of the chart – ADR details must be transferred to all new medication charts that are commenced.
- If the ADR meets the criteria as a clinical alert as outlined in [Appendix 1](#) – then a Clinical Alert Notification form (MR Alert 2) is to be completed and forwarded to the Manager Health Information Management Service (HIMS).
- Clinicians must also ensure that ADR and allergy information is included on the Notifications and Clinical Summaries (NaCS) system.

See [Appendix 3](#) for flowchart.

Consideration of further requirements for ADR notification:

- An Advisory Committee on the Safety of Medicines (ACSOM) form may need to be completed and forwarded to the Pharmacy Department and the Therapeutic Goods Administration (TGA).
- Alternatively, online reporting can be done through the TGA website. Reports of suspected adverse drug reactions can be made:
 - Online at 'Report a Problem'

- Using a 'Blue Card' available from the TGA's Office of Product Review (1800 044 114 or adr.reports@tga.gov.au) or downloaded from the TGA website at 'Blue Card' adverse reaction reporting form, or MIMS Online.
- Adverse drug reactions meeting status of serious adverse drug reaction as outlined in the WA Clinical Alert (MedAlert) Mandatory Policy should be reported to the TGA – ACSOM.
- In event of an adverse drug reaction where agent is not clearly identified, clarification from Immunology (where available) and documentation of all drugs the patient is prescribed must be reported to the TGA – ACSOM.

3.3 Rechallenge of drug

The following process should be adhered to if rechallenging a drug:

- If a drug has been prescribed and there is documentation to state a previous adverse drug reaction has occurred, the nurse/midwife must check with the prescriber that the medication is safe to administer.
- If a previous adverse drug reaction has been identified and documented appropriately, and there is a clinical need to rechallenge the patient due to no other therapeutic option being available and/or lack of clarification of the reaction, the adverse drug reaction must be acknowledged by the medical officer and reasons for rechallenge must be documented in the healthcare record.
- If the reaction is a serious adverse drug reaction (e.g. anaphylaxis, angioedema, bronchospasm, rash etc.) the team consultant must review the order and document in the patient's healthcare record if the drug is required to be administered.
- Prior to the administration of a drug rechallenge to the patient, the prescriber must ensure that there is a shared decision-making discussion had with the patient and/or carer. This should include:
 - Communication of risks and benefits of the drug rechallenge
 - Opportunity for the patient and/or carer to ask questions and raise concerns; and
 - Documentation of the patient and/or carers preferences and concerns in the healthcare record
- If found to no longer be an ADR a Clinical Alert Notification form (MR Alert 2) is to be completed requesting that the alert be deleted; and forwarded to the Manager HIMS for processing.

4. Antimicrobial allergy

Antimicrobials are substances that are safe for human use and that are used to treat or prevent infections caused by microorganisms. Antimicrobial agents include antibiotics, antifungals and antivirals. Under quality statement 3 of the 2020 Antimicrobial Stewardship clinical Care standard ([Antimicrobial Stewardship Clinical Care Standard \(safetyandquality.gov.au\)](https://www.safetyandquality.gov.au/antimicrobial-stewardship-clinical-care-standard)) there are essential elements for assessing an adverse reaction to a medicine.

- Patient's description of the event (what happened to the patient)
- Nature of the reaction (diagnosis of the reaction)
- Active ingredient(s) thought to have caused the adverse reaction
- Assessment of likelihood (certainty it was caused by the active ingredient)

- Severity of event (consequence to the patient, for example hospitalisation)
- Date and location of the care (because the original record may have
- more detail).

Armadale hospital has an antimicrobial allergy DE labelling service. This service is a jointly run between Pharmacy and Infectious Diseases. The aim of this service is to provide accurate allergy labels and improve prescribing services.

It is **recommended** that patients with a history of allergy, be referred to the service to be appropriately labelled.

5. Dietary / food allergy

A food allergy reaction can occur after eating a small amount of food. It is the responsibility of the treating medical team to ensure a clinical alert is recorded for patients with a food allergy – see Appendix 6 of the [WA Clinical Alert \(MedAlert\) Policy](#) for the listing of food allergies to be reported as a clinical alert.

Food intolerance should not be listed as a clinical alert; however, may be listed on the inside cover of the patient's healthcare record.

Where a serious dietary/food allergy has been identified during a hospital admission the treating clinician is to:

- Complete a Clinical Alert Notification form (MR Alert 2) and forward to the Manager HMS for processing.
- A food allergy clinical alert (D.Other.10), must include in the comments field the words either 'unconfirmed' or 'confirmed' depending on what follow up has been completed in determining the association between the allergy symptoms and the suspected food allergen.
- A 'confirmed' food allergy must be diagnosed by an immunology or allergy specialist.
- Add information into the iSOFT Clinical Manager (iCM) system under the 'Diet' and 'Diet Notes' sections of the Handover tab; with verbal notification to the kitchen staff to make them aware.

6. Medical condition alerts

Medical condition alerts include anaesthetic alerts, implanted devices, and non-drug related alerts (e.g. latex allergy). For a comprehensive list of medical condition alerts please see the reverse side of the Clinical Alert Notification form (MR Alert 2).

6.1 Patients presenting with an identified clinical alert in webPAS

- The admitting clerk is to print a copy of the webPAS alerts screen for all patients with a known clinical alert. An Alert sticker is to be attached to the top right-hand corner of the printed alert document and attach the document to the healthcare record.
- Supply the patient with a red patient identification band – no other coloured patient identification band is to be used. Provide an explanation to the patient and/or carer regarding the rationale for applying the red arm band.
- Clinicians are to document information on the HMC and apply alert stickers to drug and fluid charts where applicable.

6.2 Identification of new clinical alert

- Clinicians are to report clinical alerts using the Clinical Alert Notification form (MR Alert 2) – this form must be forwarded to the Manager HMS.
- Clinical coding staff shall initiate a Clinical Alert Notification (MR Alert 2) form if a clinical alert is identified during coding and forward this to the Manager HMS.

7. Advance health directives

When a patient presents to hospital (whether as an emergency presentation or a direct admission) the following process is to be followed by the treating clinician:

- Identify if the patient has an Advance Health Directive (AHD) and obtain a copy of the document
- The copy of the patient's AHD is to be forwarded to the Manager HMS along with the healthcare record so that an AHD alert (M11.02) can be added to webPAS
- The presence of an AHD clinical alert on webPAS provides a flag that identifies that an AHD exists for the patient – the details of the AHD are not stored on webPAS.

Where a patient has presented to hospital or outpatient clinic and an AHD clinical alert is identified in webPAS the treating clinician:

- Should discuss the AHD with the patient or the family/carer to determine the content and currency of the document
- It is important that the treating team be familiar with the patient's AHD and their treatment wishes. As soon as possible an advance care planning discussion should be held with the patient to clarify if they remain comfortable with their choice of preferred treatment decision(s).

Please refer to the [Advance Care Planning](#) website for further information.

8. Management of clinical alerts

The following section outlines the process for the management of medical condition and drug-related clinical alerts (including dietary/food allergies). See [Appendix 4](#) for flowchart:

8.1 Authorisation of clinical alerts

- The Manager HMS is responsible receiving all Clinical Alert Notification forms (MR Alert 2) and coordinating the entry of the clinical alerts into webPAS. Any medical condition and drug-related clinical alerts will be reviewed in a timely manner; with the exclusion of anaesthetic alerts as these will be delegated for review to the relevant responsible officer.
- A designated Anaesthetist allocated the responsibility for the governance of anaesthetic alerts will review all related clinical alerts in a timely manner and provide a response to the Manager HMS regarding the approval or non-approval of the entry into webPAS.
- The Manager HMS to review and enter all clinical alert requests into webPAS – the form is then to be filed under the Alert Divider in the healthcare record.

8.2 If the proposed alert is not approved for entry onto webPAS

- If the alert is not deemed to be an appropriate clinical alert (i.e. does not meet the criteria outlined in [Appendix 1](#)) but is still required to be captured in the patient's healthcare record, the Manager HMS should ensure the alert (medical, anaesthetic or drug alert information) is documented on the inside cover of the healthcare record as appropriate.

- Cross the Clinical Alert Notification (MR Alert 2) form through with two diagonal lines and state the reason why not approved. The MR Alert 2 form should still be filed as a record of proposed clinical alert with reason why it was not approved, in the Alerts section of the patient's healthcare record.

8.3 Removal of a clinical alert from webPAS

- If a clinical alert is no longer active, or is no longer relevant, the clinician identifying this must update the Clinical Alert Notification form (MR Alert 2) with the rationale for the removal of the clinical alert and forward to the Manager HMS.
- The Manager HMS will review the form and inactivate the clinical alert in webPAS.

9. Process for clinical alert queries

For any queries raised by the Manager HMS or a clinician with regards to the inclusion of a clinical alert in webPAS the following process should be followed:

- Queries from clinicians regarding a clinical alert should be submitted to the Manager HMS for coordination of a response.
- The Manager HMS will liaise with the Director Clinical Services and/or relevant responsible officer for the review of any medical condition and drug-related clinical alerts.
- If the clinical alert is subsequently approved then the process in [Section 7.1](#) is to be followed and the decision communicated to the clinician raising the query, where applicable.
- If the clinical alert is not approved following resolution of the query the process in [Section 7.2](#) is to be followed and the decision communicated to the clinician raising the query, where applicable.

Clinical alert coding considerations:

- Where a clinical alert for a patient does not fit within one of the specified categories the Medical Other (M. Other.4) or Drug Other (Drug.Other.3) category may be used.
- The M. Other. 4 / Drug.Other.3 alerts must only be used for clinical alerts that are potentially life-threatening conditions that are crucial for treating physicians to be alerted to immediately on patient presentation to hospital.
- D.Other.3 alerts must have the specific drug and the reaction documented before being submitted for input to WebPAS.

10. Roles and responsibilities

Role	Responsibility
Director Clinical Services	• Review and provide advice on any queries raised regarding medical condition and drug-related clinical alerts; and provide an agreed resolution to the Manager HMS for data entry into webPAS.
Manager Health Information Management Service	• Coordination of the entry of notified clinical alerts into the webPAS in a timely manner

Role	Responsibility
	• Liaison with the relevant responsible officer, for anaesthetic alerts and clinical alert queries, for review and follow-up prior to data entry into webPAS • Coordination of the removal of clinical alerts from webPAS on receipt of the appropriate authorisation • Reporting compliance with the requirements of this procedure as required by the information included in the Compliance, Monitoring and Evaluation section of this procedure.
Clinicians	• Screen patients on initial presentation to the hospital to determine the presence of any allergy and/or medical conditions that may require notification as a clinical alert • Determine the clinical importance of an allergy, adverse drug reaction or medical condition in accordance with Appendix 1 of this procedure • Complete a Clinical Alert Notification form (MR Alert 2) in accordance with the requirements of this procedure
Pharmacist	For all previous and new ADR: • Inform the patient of their adverse drug reaction and provide a copy of the Adverse Drug Reaction Information Brochure • Encourage the patient to discuss the purchase of a 'MedicAlert' bracelet, where appropriate, with their primary care provider or community pharmacy post discharge • Complete a Clinical Alert Notification form (MR Alert 2) in accordance with the requirements of this procedure For all other drug alert/documentation related queries: • Review and approve/reject alerts as identified or as requested • Copy ADR information to new volumes created as requested by clerical staff.
Clerical staff	• Add an Alert divider to the patient's healthcare record for patients with an identified ADR/clinical alert • File the Patient Alert form (MR Alert 1) and any Clinical Alert Notification form (MR Alert 2) in the patient's healthcare record according to the order of filing. • Add the appropriate Alert label to the front cover of the healthcare record (e.g. drug alert, latex alert, alert (generic)) • If the alert is not already noted on the inside cover of the health record, add the label and contact the pharmacist or clinician and advise that an alert has been identified.

Role	Responsibility
	- When creating new volumes of health records, ensure that all alerts are flagged on the new volume as well (outside cover and inside cover as above with assistance from clinical staff). - On patient attendance to the hospital print all webPAS alerts, add Alert sticker to page and attach to patient's current documentation.
Clinical Coding staff	- Identify records with important alerts that have been documented but not flagged on the healthcare record/webPAS. - Complete the Clinical Alert Notification form (MR Alert 2) and forward it to the pharmacist or appropriate clinician for authorisation. - Once authorised, forward to the Manager HIMS for entry into webPAS – AHS only. - At Kalamunda Hospital – the Clinical Coder will enter all alerts into the webPAS.

11. Legislative context

The following documents are required to give effect to this procedure:

- [Health Department WA \(HDWA\) Clinical Alert \(MedAlert\) Policy MP 0053/17](#)
- [East Metropolitan Health Service \(EMHS\) Patient Identification and Procedure Matching Policy \(EMHS: 21\)](#)

12. Supporting information

The following documents support the implementation of this procedure:

- [Advance Health Directive and Enduring Power of Guardianship Guideline \(AKG-GUI-0013-18\)](#)
- [Methicillin Resistant Staphylococcus Aureus \(MRSA\): Infection Prevention and Management Procedure \(AKG-PRO-0270-17\)](#)
- [EDIS External Register Alert Management Procedure \(EMD-PRO-0143-18\)](#)
- [Clerical Admission Procedure \(COR-PRO-0509-19\)](#)
- [Psychiatric Services Online Information System Alerts Process \(v0.5\)](#)

13. Compliance, monitoring and evaluation

Compliance against this procedure will be in accordance with the mandatory requirements set out in the [WA Clinical Alert \(MedAlert\) Policy MP 0053/17](#) and is the responsibility of the Manager HIMS.

Mandatory indicators for reporting to the EMHS Safety, Quality and Consumer Engagement team by 15 September for the preceding financial year include:

- Percentage (including numerator/denominator information) of alerts that were reviewed as appropriate to be entered onto WebPAS

- Percentage (including numerator/denominator information) of medical, pharmacy and clerical staff who have received education on clinical alerts.

Other agreed quality indicators for monitoring via the organisation-wide audit program:

- Average time for alerts to be uploaded onto WebPAS i.e. number of days from date of generating MR Alert 2 form (filled out to date entered onto PAS).
- Percentage of patient healthcare records with MR Alert 2 form that has been entered onto PAS within 48 hours of receipt of a Clinical Alert Notification form (MR Alert 2) from a 50% sample.
- Percentage of patients with a clinical alert that has not been notified (e.g. patient has had an anaphylactic reaction to medication which has been documented in healthcare record (admission notes or medication chart) but has not been raised as a clinical alert on an MR Alert 2 form).

This information is to be tabled at the Clinical Safety and Quality Committee prior to submission to the EMHS Safety, Quality and Consumer Engagement team.

14. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 4 – Action 4.7: The health service organisation has processes for documenting a patient's history of medicine allergies and adverse drug reactions in the healthcare record on presentation.
- Standard 4 – Action 4.8: The health service organisation has processes for documenting adverse drug reactions experienced by patients during an episode of care in the healthcare record and in the organisation-wide incident reporting system.
- Standard 4 – Action 4.9: The health service organisation has processes for reporting adverse drug reactions experienced by patients to the Therapeutic Goods Administration, in accordance with its requirements.
- Standard 5 – Action 5.12: Clinicians document the findings of the screening and clinical assessment processes, including any relevant alerts, in the healthcare record.
- Standard 5 – Action 5.17: The health service organisation has processes to ensure that current advance care plans:
- Standard 6 – Action 6.11: The health service organisation has processes to contemporaneously document information in the healthcare record, including:

15. References

1. Australian Commission for Safety and Quality in Healthcare: Specifications for a standard patient identification band. ACSQHC, 2008.

16. Glossary

The following definitions are relevant to this policy.

Term	Definition
Adverse drug reaction	A harmful or undesirable effect associated with the exposure to a medication/drug at therapeutic or sub-therapeutic doses.

Term	Definition
Clinical alert	A diagnosis which has the potential to be of critical importance to patients' management during the first 24 hours of their admission to hospital and assumes that the patient is not always capable of communicating such information. There are three classifications of clinical alerts; anaesthetic, medical and drug alerts. By raising a Clinical Alert (also known as MedAlert) for approval to be entered onto WebPAS, critical clinical information can be immediately flagged from the PAS for notification to clinicians before the healthcare record is retrieved.
Clinician	Clinicians include doctors, nurses, pharmacists and allied health professionals.
Serious drug reaction	A serious adverse drug reaction is defined reaction that may lead to a life-threatening event and has an absolute or relative contraindication to repeat administration of the drug.

17. Policy owner (relating to these procedures)

Director Clinical Services

Enquiries relating to this procedure may be directed to:

Title: Manager Health Information Management Service
 Department: Health Information Management Service
 Email: tanya.miocevich@health.wa.gov.au

18. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
1.2	12/01/2023	01/09/2025	Minor amendments

19. Authorisation

This procedure has been authorised and issued by the Director Clinical Services Armadale Kalamunda Group.

Authorised by	Alison Parr – A/Executive Director
Authorisation date	12/01/2023
Published date	13/01/2023

Appendix 1 – Serious life-threatening reactions for inclusion on the PAS

Only serious life-threatening reactions are to be documented on the PAS.

Both the drug implicated and the reaction which occurred MUST be specified.

A serious adverse drug reaction is defined as an absolute or relative contraindication to repeat administration of the drug. There is a need to differentiate between serious and severe reactions – “severe” is often used to describe the intensity of a medical event. Other cases require further clarification.

Medications /drugs of concern are those likely to be given without verbal consultation with the patient (i.e. when the patient is too unwell). Examples include antibiotics, anaesthetics, and analgesics.

Allergic reactions for inclusion:

(Drug and Non-Drug Allergies e.g. Latex, Intravenous Contrasts, and Chlorhexidine):

- Rash – if thought to be serious or severe, or accompanied by swelling of the whole body (not localised)
- Anaphylaxis or Anaphylactoid reactions
- Serum Sickness
- Angioedema - swelling of face, throat, neck, tongue
- Bronchospasm, asthma, other breathing difficulties.

Other serious or life-threatening reactions for inclusion:

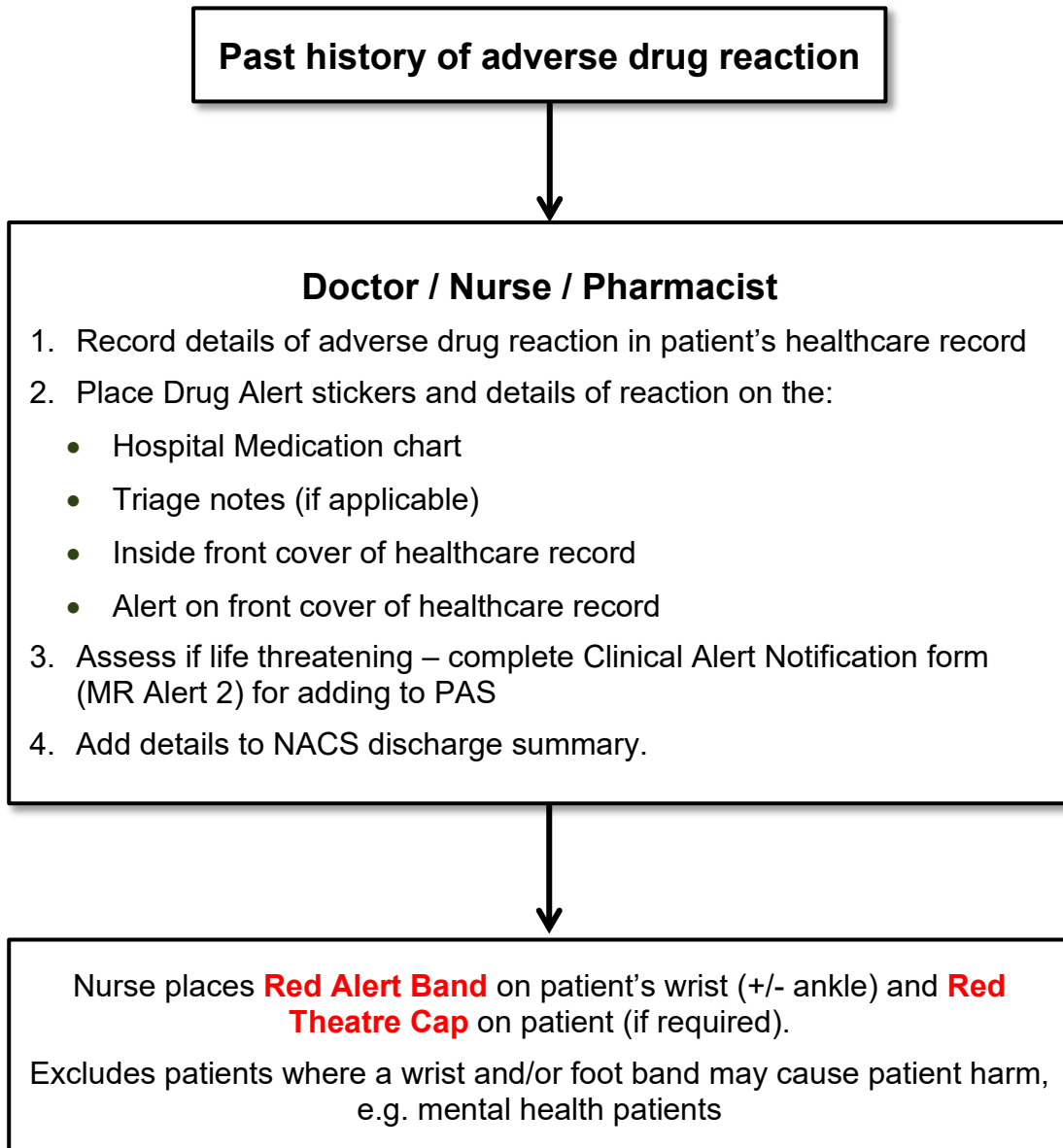
- Agranulocytosis (e.g. clozapine)
- Extrapyramidal side effects (severe dystonia / laryngospasm) to antipsychotics
- Stevens Johnson Syndrome
- Toxic epidermal necrolysis
- Malignant hyperthermia
- Suxamethonium apnoea or cholinesterase problem
- Neuroleptic Malignant Syndrome
- Hepatitis or Nephritis
- Other – must be deemed serious and life-threatening/causing significant harm.

Adverse drug reactions that are NOT deemed Clinical Alerts/Med Alerts:

- Non-dose Related Reactions – Unpredictable and uncommon side-effects not related to pharmacological action, with a low mortality rate (e.g. Lithium induced neutropenia)
- Time-related Reactions – Uncommon reactions which are usually dose-related, and occur sometime after the use of the drug (e.g. Tardive dyskinesia secondary to antipsychotic drugs)
- Dose Related Reactions – Predictable side-effects related to pharmacological action of drugs (e.g. moderate extrapyramidal side-effects to antipsychotic drugs)
- Mild to moderate side-effects or unknown reactions are not to be recorded as a clinical alert and do not require a red ID band; though these should be documented in the healthcare record e.g. mild diarrhoea, nausea and mild vomiting, itch, hayfever / blocked nose, local swelling or pain; and non-serious adverse reactions to non-drug allergens (e.g. bee stings, grasses).

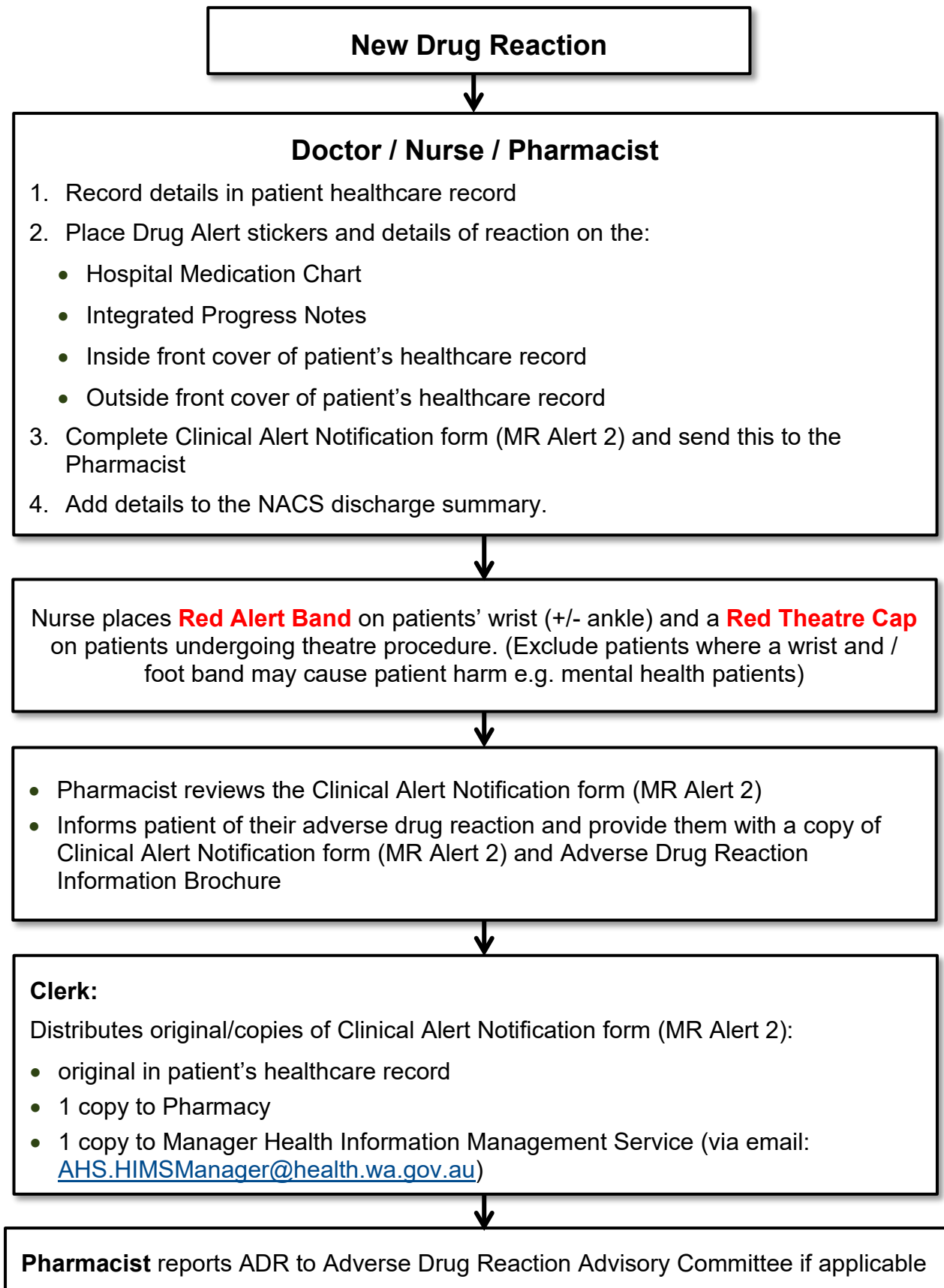
Appendix 2 – Process for patient with previous adverse drug reaction

Please see the Adverse Drug Reaction and Clinical Alert Procedure for further details.



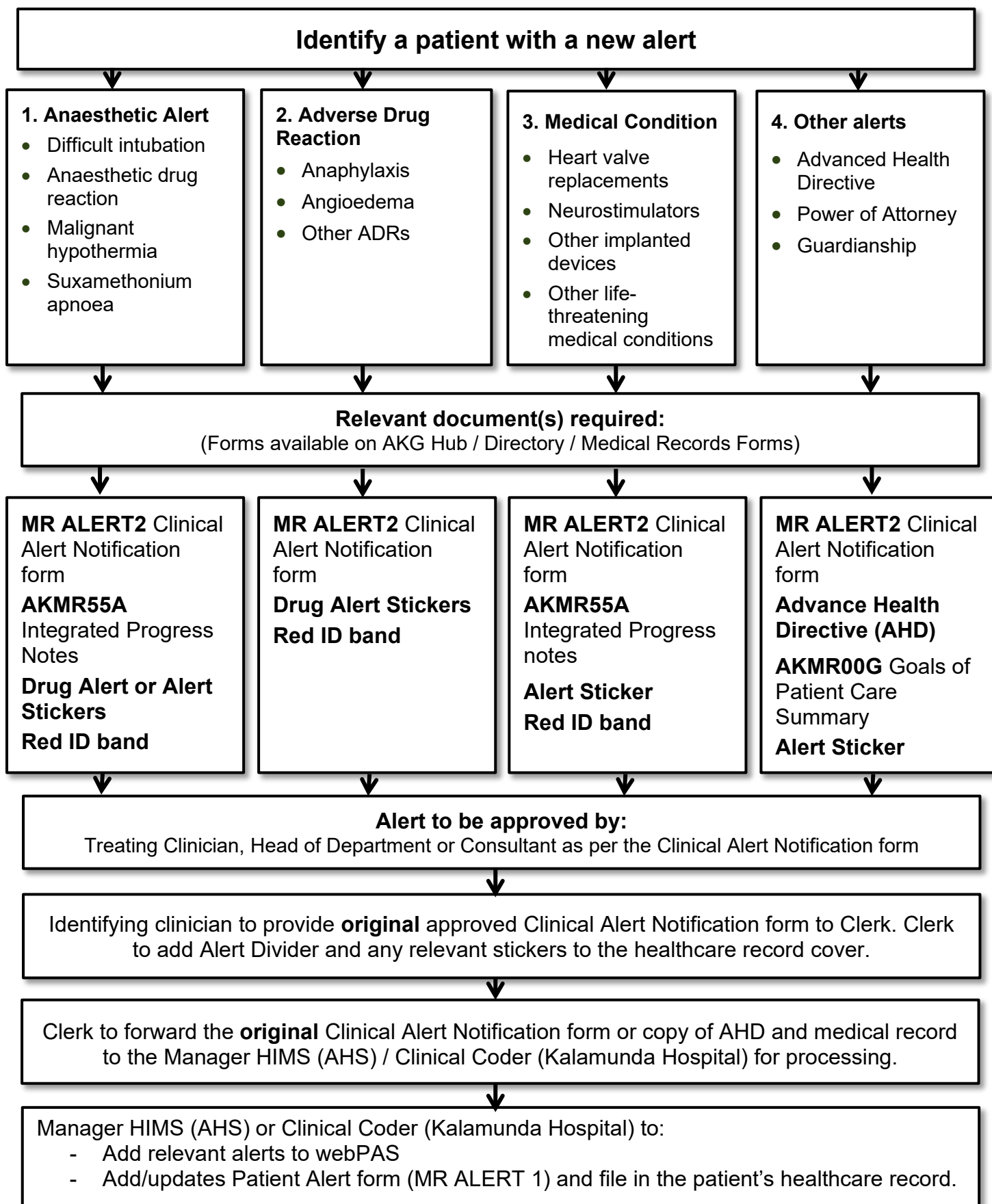
Appendix 3 – Process for patient with a new adverse drug reaction

Please see the Adverse Drug Reaction and Clinical Alert Procedure for further details.



Appendix 4 – Adverse drug reaction and clinical alert documentation

Please see the Adverse Drug Reaction and Clinical Alert Procedure for further details.

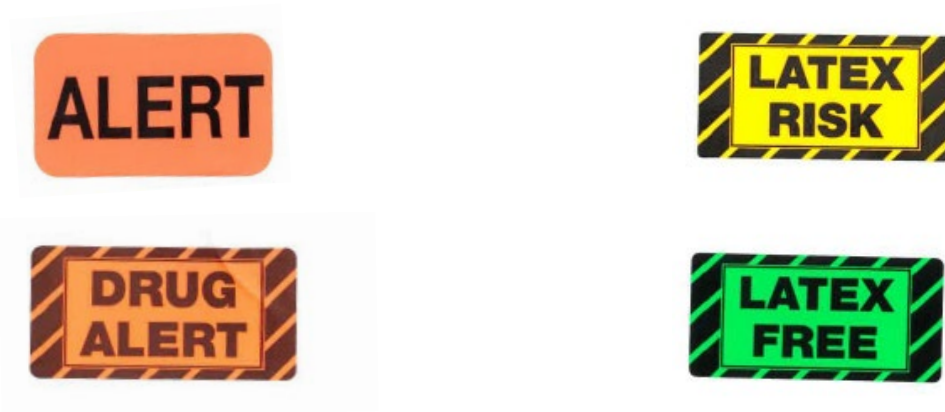


Appendix 5 – Alert Stickers

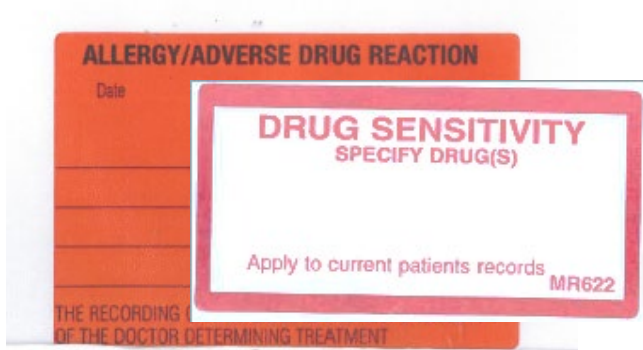
Please see the Adverse Drug Reaction and Clinical Alert Procedure for further details.

The following are examples of the alert stickers used across the Armadale Kalamunda Group:

Outside cover of the healthcare record



Inside cover of healthcare record



Healthcare record charts (e.g. Hospital Medication Charts)

